evaluates the whole experience according to their average. This has far-reaching impact on all our practical decisions. Kahneman began investigating the experiencing self and the narrating self in the early 1990s when, together with Donald Redelmeier of the University of Toronto, he studied colonoscopy patients. In colonoscopy tests, a tiny camera is inserted into the guts through the anus, in order to diagnose various bowel diseases. It is not a pleasant experience. Doctors want to know how to perform the test in the least painful way. Should they speed up the colonoscopy and cause patients more severe pain for a shorter duration, or should they work more slowly and carefully?

To answer this query, Kahneman and Redelmeier asked 154 patients to report the pain during the colonoscopy at one-minute intervals. They used a scale of 0 to 10, where 0 meant no pain at all, and 10 meant intolerable pain. After the colonoscopy was over, patients were asked to rank the test's 'overall pain level', also on a scale of 0 to 10. We might have expected the overall rank to reflect the accumulation of minute-by-minute reports. The longer the colonoscopy lasted, and the more pain the patient experienced, the higher the overall pain level. But the actual results were different.

Just as in the cold-water experiment, the overall pain level neglected duration and instead reflected only the peak-end rule. One colonoscopy lasted eight minutes, at the worst moment the patient reported a level 8 pain, and in the last minute he reported a level 7 pain. After the test was over, this patient ranked his overall pain level at 7.5. Another colonoscopy lasted twenty-four minutes. This time too peak pain was level 8, but in the very last minute of the test, the patient reported a level 1 pain. This patient ranked his overall pain level only at 4.5. The fact that his colonoscopy lasted three times as long, and that he consequently suffered far more pain on aggregate, did not affect his memory at all. The narrating self doesn't aggregate experiences – it averages them.

So what do the patients prefer: to have a short and sharp colonoscopy, or a long and careful one? There isn't a single answer to this question, because the patient has at least two different selves, and they have different interests. If you ask the